Royal Devon University Healthcare NHS Foundation Trust

Standard Operating Procedure for The Distraction, Immobilisation and Restraint of Children for Imaging Purposes; Northern Services.	
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Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

Monitoring Information		**Strategic Directions – Key Milestones**	
Patient Experience	✓	Maintain Operational Service Delivery	✓
Assurance Framework		Integrated Community Pathways	
Monitor/Finance/Performance		Develop Acute Services	
CQC Fundamental Standards Regulations No:		Delivery of Care Closer to Home	
		Infection Control	
Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

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0.1	May 2017	Radiology Governance Lead	Initial version for consultation
0.2	Oct 2017	Radiology Governance Lead	Amendments to text
1.0	Nov 2017	Radiology Governance Lead	Ratified by Radiology Management Group
2.0	June 2020	Radiology Governance Lead	Reviewed and updated: 6.2 change to play specialist details; 6.3 lead shielding removed due to change in National guidance.
2.1	Sept 2023	Radiology Governance Lead	Reviewed and updated to new Trust format minimal changes to grammar and layout made.
3.0	Sept 2023	Radiology Governance Lead	Approved by Clinical Lead for Radiology on behalf of Radiology Management Group via Q-Pulse

Associated Trust Policies/ Procedural documents:	*Standard Operating Procedure for Children Attending Radiology; Northern Services* Safeguarding Children and Young People Policy
Key Words:	Children, distraction, immobilisation, sedation
In consultation with and date: Clinical Lead Radiology – May 2017 Radiologists – May 2017 Radiographers – May 2017 Paediatric Practitioner Referrers – May 2017 Paediatric Play Specialist – May 2017 Radiology Management Group – Nov 2017 & Oct 2023	
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KEY POINTS OF THIS PROCEDURAL DOCUMENT:

Radiographers and other health professionals involved in imaging should be clear in their duty of care; acting in the best interests of the child during diagnostic imaging. The restraint of children, or restricting their movements in order to perform diagnostic imaging should always be very carefully considered.

Co-operation and Distraction is always preferable to restraint and immobilisation in all cases.

If immobilisation is necessary then “gentle, protective restriction of movement using pads and other devises to maintain position is acceptable” as long as the child and parent/guardian have been adequately informed and prepared in advance (SOR, 2005).

1. INTRODUCTION

1.1. The imaging of children is often challenging. Dependant on age they are often unable to fully co-operate with Radiographers and other Health Professionals during diagnostic imaging. This may be due to age, lack of understanding, pain, distress, or fear of the unknown and the equipment, environment and staff.

1.2. According to the Society of Radiographers (2012) "Gentle immobilisation is required to obtain images of sufficient quality and the exact technique may vary in individual cases, the fact that the child was not cooperative is no excuse for the production of an inferior quality image".

1.3. Radiographers and other health professionals involved in imaging should be clear in their duty of care; acting in the best interests of the child during diagnostic imaging. The restraint of children, or restricting their movements in order to perform diagnostic imaging should always be very carefully considered by the Radiographer.

2. PURPOSE

2.1. The Standard Operating Procedure (SOP) has been written to:

- Ensure that the care and treatment given is lawful, reasonable and proportionate, as well as the least restrictive pathway available.
- Clarify the roles and responsibilities of the radiographer.
- Guide the use of explanation, consent and distraction.
- Guide the use of immobilisation.
- Guide the use of sedation.
- Inform education and training.

3. DEFINITIONS

3.1. NDDH – North Devon District Hospital
3.2. MRI – Magnetic Resonance Imaging
3.3. IR(ME)R – Ionising Radiation (Medical Exposures) Regulations 2017

4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1. The Radiology Governance Lead is responsible for the implementation of this standard operating procedure and monitoring its compliance.
4.2. All Staff have a personal and professional duty to safeguard any child and ensure that they do not suffer harm especially whilst in the radiology department
4.3. All Staff must follow the Trust's Safeguarding Children and Young People Policy and act appropriately if they have concerns of abuse and/or neglect of a child.

4.4. **All Staff** imaging children at these sites must be up to date in their training on child protection and know their responsibilities in undertaking imaging of children for diagnostic, clinical or evidential purposes.

4.5. All Staff using radiation in the course of their work to be appropriately trained and made aware of the need to adhere to the relevant regulations

5. PROCEDURE

5.1. Paediatric Equipment Available

5.1.1. There is a paediatric distraction light tower kept in the general department in room 3 that can be used for all procedures (except MRI), and can be easily moved to other areas as necessary.

5.1.2. Each room has soft positioning pads for positioning and support of patients along with other effective immobilisation devises and distraction aids.

5.2. Duty of Care.

5.2.1. During imaging procedures, it is essential that the radiographer ensures that the care and treatment given is lawful, reasonable and proportionate, and the least restrictive pathway available is used.

5.2.2. Consent is often assumed in radiography by the informed patient coming into the room and lying on the bed or standing in position for imaging. However, verbal consent should always be sought and given freely for paediatric procedures from either the patient if legally competent and able to do so, or their accompanying parent/guardian following a full age appropriate explanation of the procedure.

5.2.3. In cases of Non-accidental Injury skeletal surveys written consent should be sought from the parent or guardian consenting to the examination (see Local protocols for NAI skeletal surveys).

5.3. The Roles and Responsibilities of the Radiographer.

5.3.1. All radiographers must ensure that they act in the best interests of their patients, this duty of care is essential in undertaking the imaging of children.

5.3.2. The radiographer is responsible for the preparation of children and the undertaking of their diagnostic imaging procedures. It is essential that consent is obtained and that the child, in as much as they are able to, understands what the procedure will entail, how it will be done and what they are expected to do during the procedure. If consent is refused this should be recorded along with the reasons given.

5.3.3. The radiographer must be mindful of the language that s/he uses to describe the examination to the child and ensure that it is age appropriate to ensure clear understanding and co-operation.

5.3.4. Co-operation and Distraction is always preferable to restraint and immobilisation in all cases. The need for restraint may be avoided through clear explanation, active encouragement of the child, gaining the confidence of the

child and parent/guardian, through the use of constructive play and the use of distraction techniques that are age appropriate.

5.3.5. It is advisable to enlist the assistance of the Play Specialist (Caroline Thorpe Ward) to provide support when necessary; this is essential for NAI cases.

5.4. The use of Immobilisation.

5.4.1. The Society of Radiographers (SOR) (2005) state "Restraining children from moving or restricting their movement in order to carry out a diagnostic examination should be carefully considered by the radiographer. The radiographer should weigh up the situation and anticipate the possible need for the person in attendance [preferably a parent] to hold the child still for the procedure whilst at the same time give consideration of ways to prevent the need to restrain".

5.4.2. If immobilisation is necessary then "gentle, protective restriction of movement using pads and other devises to maintain position is acceptable" as long as the child and parent/guardian have been adequately informed and prepared in advance (SOR, 2005).

5.4.3. Radiographers should be mindful of the amount of 'force' used to restrict movement and be familiar with the legal and ethical implications of restraining children in this manner as well as local trust policy.

5.4.4. It is recognised that any child may show distress when immobilisation is used, however again radiographers should be mindful of immobilising children who may have been physically abused to avoid further psychological damage.

5.4.5. A selection of immobilisation devises are available in all x-ray rooms such as sponge wedges, sand bags, Perspex blocks, Velcro bands and micropore tape.

5.4.6. The Paediatric Distraction Light Tower is also available to provide distraction through lights, music, visual displays and play as well as a selection of teddies and toys (kept in room 3).

5.5. The use of Sedation.

5.5.1. Sedation may be necessary for some examinations where the child has become distressed and is unable to continue (particularly for skeletal surveys). However, this should always be decided by the clinical team following careful and appropriate assessment of the child.

5.5.2. If an examination needs to be abandoned due to the distress or lack of co-operation of the child, or withdrawal of consent from the child or their parent/guardian then this should be documented and the child referred back to the referring clinician.

5.6. Education and Training.

5.6.1. All staff undertaking imaging of children must be up to date with all mandatory training, particularly child protection modules

5.6.2. Radiographers must maintain up to date CPD and ensure current knowledge of legislation and medico-legal issues relating to the imaging of children.

6. ARCHIVING ARRANGEMENTS

6.1. The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely.

6.2. This document will be stored electronically on the radiology G:drive here and maintained on Q-Pulse

6.3. Archived electronic copies will be stored in the obsolete documentation/archive folder here.

6.4. All paper copies past the review date will be destroyed with only the electronic obsolete copy available (as above).

7. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE STANDARD OPERATING PROCEDURE/ GUIDELINE

7.1. To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Local compliance with policy	Internal audit	Audit reported to the Radiology Management Group and at Radiology Audit meeting
Positive assurance of compliance	Minutes of Meetings	Monitored by Radiology Management Group
Related Incidents	Minutes of meetings	Monitored via Specialty Governance Group

8. REFERENCES

- Society of Radiographers (2005) The Child & the Law: The Roles & Responsibilities of the Radiographer. Available from: http://www.sor.org/system/files/article/201202/sor_child_law_roles_responsibilities.pdf

- Society of Radiographers (2012) Imaging Children; immobilisation, distraction techniques and use of sedation. Available from: http://www.sor.org/learning/document-library/imaging-children-immobilisation-distraction-techniques-and-use-sedation

- Standard Operating Procedure for Children Attending Radiology; Northern Services

- Safeguarding Children and Young People Policy

APPENDIX X: COMMUNICATION PLAN

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The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the guideline/SOP	Divisional Directors / General Managers Radiology Management Group All Staff working with or supporting children in the radiology department
The key changes if a revised document	Reviewed and updated to new Trust format minimal changes to grammar and layout made.
The key objectives	To: • Ensure that the care and treatment given is lawful, reasonable and proportionate, as well as the least restrictive pathway available. • Clarify the roles and responsibilities of the radiographer. • Guide the use of explanation, consent and distraction. • Guide the use of immobilisation. • Guide the use of sedation. • Inform education and training.
How new staff will be made aware of the procedure/guideline and manager action	Cascade by email from Radiology Governance Lead, induction process, team meetings, radiology newsletters, notification from Q-pulse
Specific Issues to be raised with staff	Staff should be made aware of the policy. Staff should be aware of their responsibilities for safeguarding children.
Training available to staff	Support and advice is available from Radiology Governance Lead and Paediatric Lead Radiographers
Any other requirements	Completion of mandatory training for Safeguarding Adults and Children

Issues following Equality Impact Assessment (if any)	N/A
Location of hard / electronic copy of the document etc.	Policy is on the Trust Intranet – Northern Services This document will be stored electronically on the radiology G:drive and maintained on Q-Pulse